

HEALTHCONNECT CLINIC EXPERIENCE LAB

WEEK 5 — TRACK COORDINATION PLAN

Improving Patient Appointment Attendance and Healthcare Support Using Data and AI

Document Control

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1. PURPOSE

The Track Coordination Plan establishes the control framework through which HealthConnect workstreams coordinate their interfaces, dependencies, information exchanges, handoffs and cross-track decisions during execution.

Its purpose is to ensure that progress within individual workstreams translates into coordinated project progress. The plan defines how outputs become downstream inputs, how readiness is established, how interface constraints are managed and how matters exceeding individual workstream authority are escalated.

This document operates alongside the Project Task Tracker, Execution & Progress Plan, Risk and Dependency Register, Issues & Decisions Log and Project Timeline. It does not duplicate their primary control functions.

2. COORDINATION OBJECTIVES

The coordination framework is intended to:

1. Maintain visibility of material interfaces between workstreams.
2. Establish clear ownership for cross-track inputs, outputs and handoffs.

3. Ensure downstream activities receive sufficiently complete and usable inputs.
4. Identify dependency constraints before they develop into material delivery issues.
5. Enable workstreams to progress in parallel where genuine dependencies do not prevent progression.
6. Maintain traceability between workstream outputs and integrated project deliverables.
7. Coordinate cross-track issues and decisions where resolution extends beyond a single workstream.
8. Protect solution quality by preventing premature downstream progression.
9. Maintain an integrated view of readiness across technical and operational workstreams.
10. Support controlled transition between execution stages and subsequent project phases.

3. COORDINATION PRINCIPLES

3.1 Interface-Based Coordination

Coordination is centered on the points where workstreams exchange information, outputs, decisions or requirements rather than on workstream activity in isolation.

3.2 Controlled Parallelism

Workstreams may progress concurrently where their activities are not constrained by unresolved predecessors. A dependency should create sequencing only where the dependent activity genuinely requires the predecessor output.

3.3 Output Readiness

Producer completion does not automatically establish downstream readiness. An output must satisfy the requirements of its intended receiving workstream before the associated dependency is treated as resolved.

3.4 Explicit Accountability

Each material interface has an identifiable producing owner and receiving owner. Responsibility for producing an output does not automatically transfer accountability for its downstream use.

3.5 Evidence-Based Coordination

Status, readiness and escalation decisions are based on available project evidence rather than assumptions, anticipated completion or informal confirmation.

3.6 Early Constraint Visibility

Potential dependency failure, conflicting requirements, resource constraints or quality concerns should be surfaced as soon as their downstream implications become reasonably identifiable.

3.7 Integrated Decision Discipline

Where a decision affects multiple workstreams, its cross-track implications must be understood before the decision is considered closed.

4. WORKSTREAM COORDINATION ARCHITECTURE

HealthConnect operates through the following coordinated workstreams. Each keeps the colour used across the HealthConnect project documents.

Project Management	Data & Analytics	Predictive Intelligence	Patient Engagement	Generative AI Support	Process Integration	Testing & Validation	Implementation / Adoption
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Workstream	Primary Coordination Interface
Project Management	Integrated coordination, dependency control, issues, decisions, escalation and schedule integration
Data & Analytics	Data assessment and attendance / no-show evidence
Predictive Intelligence	Initial no-show prediction approach informed by validated analytical findings
Patient Engagement	Evidence-informed intervention opportunities
Generative AI Support	Routine-enquiry support requirements, boundaries and escalation logic
Process & Operational Integration	Translation of solution components into clinic workflow requirements
Testing & Validation	Evidence, validation and acceptance requirements
Implementation / Adoption	Operational readiness and adoption considerations

Project Management operates across the workstream interfaces, maintaining the integrated control layer rather than forming a downstream stage in the technical delivery chain.

5. COORDINATION ACCOUNTABILITY MODEL

Coordination Area	Accountable Function	Control Responsibility
Integrated coordination	Project Management	Maintain visibility of material cross-track interfaces
Workstream delivery	Relevant Workstream Lead	Deliver agreed outputs and report current position
Output production	Producing Workstream	Ensure output is sufficiently complete and documented
Output receipt	Receiving Workstream	Confirm suitability for intended downstream use
Dependency control	Project Management	Monitor condition, ownership, exposure and downstream effect
Interface resolution	Affected Workstream Leads	Resolve matters within available authority
Cross-track escalation	Project Management	Coordinate escalation of unresolved material constraints
Cross-track decisions	Project Management / Appropriate Authority	Consolidate implications and record agreed direction
Readiness confirmation	Producing & Receiving Workstreams	Confirm required conditions for progression
Coordination records	Project Management	Maintain evidence of material handoffs, actions and decisions

6. WORKSTREAM INTERFACE MATRIX

The following interfaces represent the principal coordination points during the current execution stage.

ID	Producing Workstream	Receiving / Affected Workstream	Principal Exchange	Readiness Condition
IF-01	Data & Analytics	Predictive Intelligence	Attendance / no-show findings	Findings sufficiently established to support identification of predictive variables
IF-02	Data & Analytics	Patient Engagement	Attendance / no-show findings	Evidence sufficiently clear to inform intervention opportunities
IF-03	Data & Analytics	Project Management	Data-readiness and analytical position	Current evidence available for dependency and schedule control
IF-04	Generative AI Support	Process & Operational Integration	Support functions, boundaries and escalation requirements	Support logic sufficiently defined for workflow mapping
IF-05	Patient Engagement	Process & Operational Integration	Proposed intervention concepts	Intervention logic sufficiently defined for workflow assessment
IF-06	Predictive Intelligence	Testing & Validation	Initial predictive output	Predictive approach sufficiently defined for validation planning
IF-07	Process & Operational Integration	Testing & Validation	Integrated workflow design	Relevant workflow interfaces sufficiently specified for validation
IF-08	Testing & Validation	Implementation / Adoption	Validation and acceptance position	Evidence available to inform implementation-readiness assessment
IF-09	All Workstreams	Project Management	Progress, issues, dependencies and decisions	Current information available for integrated project control

7. DEPENDENCY AND HANDOFF CONTROL

A cross-track dependency is considered controlled when the following elements are identifiable:

- Required input or output;
- Producing workstream;
- Receiving workstream;
- Dependency condition;
- Responsible owner;
- Expected handoff;
- Receiving-side acceptance condition;
- Current status;
- Downstream consequence where the dependency remains unresolved.

Handoff Control Sequence



Where an output does not satisfy the receiving requirement, the receiving workstream records the deficiency and coordinates its resolution rather than treating the dependency as complete. Project Management maintains visibility of material unresolved handoffs where they may affect scope, schedule, quality, risk, resources or downstream readiness.

8. PRINCIPAL CROSS-TRACK INTERFACES

8.1 Data & Analytics → Predictive Intelligence

Data & Analytics → Predictive Intelligence

Predictive Intelligence requires sufficiently reliable attendance and no-show findings to identify variables relevant to prediction.

The interface therefore requires:

- Completion of the relevant analytical assessment;
- Identification of meaningful attendance / no-show patterns;
- Communication of relevant analytical findings;
- Confirmation that the available evidence is adequate for the intended predictive activity;
- Visibility of limitations that could affect subsequent modelling.

A data activity being underway does not, by itself, establish that Predictive Intelligence is fully enabled.

8.2 Data & Analytics → Patient Engagement

Data & Analytics → Patient Engagement

Patient Engagement uses attendance evidence to identify intervention opportunities.

Coordination should therefore establish that:

- Relevant attendance / no-show patterns have been identified;
- Findings are sufficiently interpretable for intervention design;
- Analytical limitations are understood;
- Intervention concepts remain traceable to available evidence.

This protects the intervention workstream from progressing on unsupported assumptions.

8.3 Generative AI Support → Process & Operational Integration

Generative AI Support → Process Integration

Generative AI Support establishes the requirements governing routine enquiry assistance, including applicable boundaries and escalation conditions.

Process & Operational Integration uses these outputs to assess how support functions can operate within clinic workflows.

The interface should therefore address:

- Supported enquiry categories;
- Knowledge-based requirements;
- Information boundaries;
- Escalation triggers;
- Human handoff requirements;
- Operational ownership.

An AI support capability is not operationally ready merely because its functional requirements have been defined.

8.4 Patient Engagement → Process & Operational Integration

Patient Engagement → Process Integration

Patient Engagement provides intervention concepts derived from attendance findings.

Process & Operational Integration assesses how those interventions can be incorporated into clinic processes.

The interface should establish:

- Intended patient action;
- Operational trigger;
- Responsible clinic function;
- Workflow implication;
- Information or system requirement;
- Exception or escalation condition.

8.5 Predictive Intelligence / Process & Operational Integration → Testing & Validation

Testing & Validation requires sufficiently defined solution components to establish appropriate evidence and acceptance requirements.

The receiving workstream should therefore confirm that:

- The predictive approach is sufficiently defined for evaluation;
- Workflow logic is sufficiently specified;
- Expected behaviour can be assessed;
- Applicable validation measures can be established.

Technical completion alone does not constitute validation readiness.

9. COORDINATION CHECKPOINTS

Coordination checkpoints are triggered by material delivery conditions rather than by meeting frequency alone.

Checkpoint	Trigger	Required Assessment	Control Output
CP-01	New cross-track dependency identified	Dependency, owner, condition and impact	Dependency position
CP-02	Workstream output becomes available	Completeness and downstream suitability	Handoff / acceptance position
CP-03	Dependency condition is not met	Cause, impact and recovery requirement	Corrective action / escalation
CP-04	Issue affects multiple workstreams	Cross-track impact and decision requirement	Issue / decision position
CP-05	Output approaches downstream readiness	Acceptance conditions and outstanding gaps	Readiness position
CP-06	Material schedule movement occurs	Dependency, milestone and downstream implications	Integrated schedule position
CP-07	Stage or weekly transition occurs	Outstanding interfaces and carry-over conditions	Transition position

10. COORDINATION REVIEW MODEL

Coordination operates through three control levels.

Level 1 — Workstream Interface

Workstream Leads monitor their own inputs, outputs, dependencies and emerging interface constraints.

Level 2 — Cross-Track Coordination

Project Management consolidates material interfaces, unresolved handoffs, blockers, decisions and dependency conditions requiring integrated attention.

Level 3 — Project-Level Escalation

Matters exceeding workstream authority or creating material exposure to scope, schedule, quality, resources, risk, requirements or project value are escalated to the appropriate project authority.

This model allows routine matters to remain close to the work while ensuring material cross-track conditions receive appropriate project-level visibility.

11. CROSS-TRACK ISSUE MANAGEMENT

A cross-track issue exists where an actual condition affects more than one workstream or prevents a required interface from progressing as intended.

Control Path



Priority should be given to issues that:

- Block downstream activity;
- Affect a material dependency;
- Compromise output quality;
- Create conflicting requirements;
- Require authority outside the affected workstream;
- Create material schedule exposure;
- Affect patient-support boundaries or responsible-use requirements.

Material issues are recorded through the project's formal Issues & Decisions control rather than being managed solely through informal coordination discussions.

12. DECISION AND ESCALATION CONTROL

A matter should be escalated where the affected workstream cannot resolve it within its authority, capability or required timeframe.

Escalation Level	Condition	Management Response
Level 1	Local workstream constraint	Resolution by Workstream Lead
Level 2	Cross-track interface constraint	Project Management coordinates resolution
Level 3	Material project-level consequence	Escalation to appropriate project authority

Material escalations should identify:

- Matter requiring resolution;
- Affected workstreams;
- Current impact;
- Available response options;
- Recommended action where appropriate;
- Decision or action owner;
- Required resolution point;
- Consequence of non-resolution.

13. CHANGE COORDINATION

A change originating within one workstream must be assessed for cross-track implications where it could affect:

- Project scope;
- Requirements;
- Deliverables;
- Workstream ownership;
- Dependencies;
- Workflow design;
- Validation requirements;
- Schedule or milestones;
- Resources;
- Risk exposure.

The originating workstream identifies the proposed change. Project Management coordinates the cross-track impact assessment where other workstreams are affected.

A change affecting a material interface should not be considered fully incorporated until affected dependencies, outputs and downstream implications have been addressed through the applicable project controls.

14. READINESS AND ACCEPTANCE CONTROL

HealthConnect distinguishes between activity completion, output availability and downstream readiness.

Condition	Coordination Interpretation
Activity commenced	Work is underway
Activity completed	Producing activity has finished
Output produced	Required artefact or result exists
Output transferred	Handoff has occurred or been initiated

Condition	Coordination Interpretation
Output accepted	Receiving workstream confirms suitability
Dependency satisfied	Downstream condition has been met
Downstream activity enabled	Receiving workstream can progress on the accepted input

This distinction prevents producer-reported completion from being interpreted as automatic downstream readiness.

15. INFORMATION EXCHANGE AND RECORD CONTROL

Cross-track information exchanged for delivery purposes should be sufficiently clear to establish:

- What is being provided;
- Why it is required;
- Which workstream owns it;
- Which workstream receives it;
- Any limitations or assumptions;
- Required action;
- outstanding clarification;
- Acceptance or rejection position.

Project Management maintains the coordination record for material cross-track exchanges. Routine workstream communication does not require separate project-level recording unless it creates a dependency, issue, decision, change or readiness implication.

This prevents the coordination mechanism from becoming an unnecessary administrative reporting layer.

16. COORDINATION EVIDENCE

Evidence of effective coordination may include:

- Confirmed workstream ownership;
- Documented handoffs;
- Accepted or returned outputs;
- Dependency updates;
- Cross-track action records;
- Issue and decision records;
- Escalation records;
- Readiness confirmations;
- Agreed changes affecting interfaces;
- Updated schedule positions resulting from verified dependency conditions.

The evidence should demonstrate not merely that communication occurred, but that coordination resulted in controlled movement of work across interfaces.

17. WEEK 5 COORDINATION POSITION

The Week 5 execution structure establishes cross-track coordination as an active project-control function. The immediate coordination requirement is to maintain visibility of the interfaces connecting analytical, predictive, patient-engagement, Generative AI and operational activities while protecting downstream work from unresolved prerequisites.

Current coordination attention is therefore directed toward:

1. Establishing reliable conditions for the Data & Analytics outputs required by downstream analytical activities.
2. Maintaining visibility of dependencies affecting Predictive Intelligence and Patient Engagement.
3. Allowing Generative AI Support activities to progress independently where their prerequisites permit.
4. Preparing the intervention and support outputs required for Process & Operational Integration.
5. Maintaining visibility of the interfaces required for subsequent Testing & Validation.
6. Ensuring cross-track issues, decisions and dependency changes are reflected in the appropriate project controls.

The coordination position remains active, with downstream progression governed by the availability, suitability and acceptance of required upstream outputs.

18. WEEK 6 TRANSITION CONTROL

At transition into Week 6, Project Management should confirm the following.

Transition Area	Required Control Position
Workstream ownership	Confirmed or outstanding assignments explicitly identified
Material dependencies	Current condition, owner and downstream effect visible
Data foundation	Current readiness position and limitations established
Attendance analysis	Current evidence position established
Predictive Intelligence	Dependency condition and next required action defined
Patient Engagement	Intervention-development position established
Generative AI Support	Requirements, boundaries and escalation position progressed
Operational Integration	Required upstream inputs identified
Testing & Validation	Initial evidence and acceptance requirements sufficiently defined
Issues & Decisions	Material open matters recorded with owners
Schedule	Integrated position reconciled against available execution evidence
Carry-over work	Outstanding interfaces and priorities clearly identified

19. COORDINATION GOVERNANCE STATEMENT

The Track Coordination Plan provides the interface-control layer through which individual workstream activity is converted into integrated project progress.

Its governing principle is:

Coordination is demonstrated not by the frequency of communication, but by the controlled movement of inputs, outputs, decisions and readiness conditions across workstream boundaries.

Project Management will therefore coordinate the interfaces rather than duplicate the workstreams, maintaining visibility of the conditions required for one track to enable another and ensuring that unresolved cross-track constraints are identified, owned and escalated appropriately.

End of Week 5 Track Coordination Plan